TCF induction followed by carboplatin and radiotherapy

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate concurrent carboplatin AUC 1.5 + radiotherapy phase · NCCP 00332

Active medicines: Carboplatin

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	PRN Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- If there is no progressive disease and marrow function is adequate, the source describes weekly carboplatin AUC 1.5 with radiotherapy for 7 weeks, followed by consideration of surgery. NCCP 00323 says start 3–8 weeks (Days 22–56) after the START of induction cycle 3. Its carboplatin cross-reference says 00322, whereas the matching current regimen is 00332. The 00332 indication text says after COMPLETION of induction, while its treatment paragraph says after START of cycle 3. Resolve the date origin and correct route with the treating team before setting patient dates.
- Confirm the separate 00332 prescription; no induction docetaxel/cisplatin/fluorouracil, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or hydration automatically carries forward.
- NCCP gives no routine G-CSF or additional supportive-care prescription for this weekly phase. Do not carry over induction G-CSF, docetaxel premedication or cisplatin hydration automatically.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00323 v7b · pNCCP 00323: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00323 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required